Atypical Presentations

Atypical presentations of molluscum contagiosum may include giant, cystic, ulcerated, follicular, condyloma acuminatum-like, sebaceous naevus-like, pyogenic granuloma-like, pseudolymphomatous, cellulitis-like, or abscess-like lesions.

An eczema-type reaction, known as molluscum dermatitis, characterized by erythema, scaling, and inflamed lesions, may occur secondary to a local immune response and often precedes clinical resolution. In contrast, molluscum contagiosum occurring in the context of atopic dermatitis (termed eczema molluscatum) can present a clinical challenge, manifesting as disseminated, inflammatory lesions covered by scales or hemorrhagic crusts and associated with pruritus.

Immunocompromised Patients

Immunosuppressed individuals are prone to developing extensive, confluent, giant, multiple, or disseminated lesions. Common causes of immunosuppression linked to molluscum contagiosum include: - HIV infection - Solid organ transplantation - Immunosuppressive therapy, including biologic agents - Systemic lupus erythematosus - Sarcoidosis - Neoplasia

Giant nodules of molluscum contagiosum may be the first clinical sign of HIV infection, may appear during immune reconstitution inflammatory syndrome (IRIS) after initiating combination antiretroviral therapy in people living with HIV (PLWH), or may occur in late-stage AIDS.

Complications

Other complications include: - Bacterial superinfection - Cellulitis - Chronic conjunctivitis and keratitis (in cases of ocular involvement)

Congenital molluscum contagiosum via vertical transmission, presenting as eyelid or scalp lesions, has also been reported.

Molluscum contagiosum is typically self-limiting, with lesions resolving spontaneously within 6 months to 5 years. Persistent infection is usually seen in immunocompromised individuals.

Diagnosis

Clinical Diagnosis

The classic clinical features are multiple skin-colored, sometimes whitish, umbilicated, non-confluent papules measuring 2–5 mm in diameter. Lesions can appear on any part of the body, including the palms and soles. Their distribution is variable and often shows clustering or linear arrangement due to autoinoculation from scratching or microtrauma.

Most affected patients are children aged 3–10 years. In adults, lesions are commonly found in the genital region, although not always acquired through sexual contact. In pediatric patients, a history of mild atopic dermatitis is frequent, with lesions preferentially developing on eczematous skin—leading to the diagnosis of eczema molluscatum.

Solitary giant molluscum lesions may occur in the inguinal or perianal regions and can be mistaken for fibroma molle (acrochordon).

Immunodeficiency is a recognized risk factor.

Diagnosis is primarily clinical. Dermoscopy and in vivo confocal microscopy can be valuable in diagnostically challenging cases—such as inflamed lesions, perilesional inflammation, or small lesions.

- **Dermoscopy** is more sensitive than visual inspection and reveals central umbilicated orifices and specific vascular patterns (crown, radial, flower, or punctiform).
- **In vivo confocal microscopy** shows round, well-circumscribed lesions with central cystic areas filled with brightly refractile material, corresponding to molluscum bodies seen on histopathology.

Laboratory Diagnosis

Laboratory testing is not routinely required.

In uncertain cases, diagnosis can be confirmed by: - **Histological examination**: Reveals characteristic "nuts in a sack" appearance due to large intracytoplasmic inclusion bodies (molluscum bodies). - **Polymerase chain reaction (PCR)**: Detects molluscum contagiosum virus DNA. - **Electron microscopy**: Identifies the typical brick-shaped virions of the molluscum contagiosum virus.